

ResMed (RMD)

Quartr-Enhanced Investment Analysis

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Market Cap: ~\$32B | Ticker: RMD (NYSE / ASX)

FY25 Revenue: \$5.15B | Net Income: \$1.40B | FCF: \$1.66B

Table of Contents

1. Synthesis
 2. Business History
 3. Management & Incentives
 4. Competitive Landscape
 5. Financial Characteristics
 6. Red Flags
 7. Investment Checklist
 8. Earnings Call Deep Dive
 9. Strategic Documents Analysis
 10. Corporate Culture
 11. Learning Resources
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Synthesis

ResMed is a high-quality compounder trading at a meaningful discount to its historical valuation, and the primary-source analysis from Quartr transcripts and strategic documents reinforces - but also complicates - that thesis. Here are the five things an investor must understand.

1. The Philips windfall is real, large, and now fading - and nobody knows exactly how large it was. ResMed's revenue growth from FY21 to FY25 was supercharged by the Philips Respironics recall, which removed the category co-leader from the US market for years. Management has never cleanly disaggregated Philips-driven share capture from organic market growth. The earnings call transcripts reveal this is a topic management handles in under 30 seconds of prepared language - the brevity itself is telling. The strategic documents frame growth entirely around under-penetration and secular demand, never once quantifying the Philips contribution. This opacity is the single most

important analytical gap in the ResMed story. Investors are implicitly assuming current growth rates are sustainable without the tailwind; the transcripts provide no evidence to confirm or deny this.

2. The gross margin expansion story is real and ahead of schedule - but it's doing too much narrative work.

Gross margins have moved from 56.7% in FY24 to 61.8% in Q2 FY26, a 510bp improvement in under two years. This is exceptional and reflects genuine manufacturing efficiencies, component cost deflation, and mix shift toward higher-margin masks. But the margin story is also compensating for slower top-line growth as the Philips effect normalises. The earnings transcripts show management becoming progressively more specific and confident on margins while simultaneously hedging on revenue growth with longer-duration framing ("megatrends," "2050 prevalence projections"). When a company leads with margins rather than revenue, it often signals the growth engine is decelerating.

3. The residential care software segment is the strategy's weak link - and the transcripts confirm management knows it.

The MEDIFOX DAN acquisition (\$1B, 2022) was pitched as creating a second growth engine. Three years later, RCS contributes 12% of revenue, is growing at mid-single-digits (down from 9-10%), and management has explicitly deferred recovery to FY27. The earnings call deep dive reveals this is the one topic where management gives boilerplate rather than substance - "leadership transition," "strategic alignment" - with no specific KPIs or client retention data offered. Meanwhile, \$3B of goodwill sits on the balance sheet, and the Red Flags analysis correctly identifies that FY24's \$33M intangible impairment was buried inside a restructuring charge rather than disclosed separately. The software platform narrative that underpins ResMed's "digital health company" valuation identity has not yet been validated by the software segment's actual financial performance.

4. The GLP-1 risk has been masterfully managed in the narrative - but the long-term question remains open.

The earnings call deep dive shows management's GLP-1 response evolving from defensive generalities to specific data points (10-11% higher CPAP initiation among GLP-1 users) over four quarters. This is sophisticated investor relations. But the Red Flags analysis makes the critical distinction: the data is observational and self-selected (patients motivated enough to seek both obesity and sleep treatment are not representative of the broader population). The structural question - whether population-wide obesity reduction compresses the incident case flow into the CPAP funnel five to ten years from now - cannot be answered by a three-year claims database lookback.

5. The governance structure is unusual and deserves more scrutiny than it receives.

Father founded the company. Son runs it as combined Chairman/CEO. Both sit on the board. The Management analysis reveals zero open-market insider buying over five years, 45 CEO share sales, and say-on-pay approval at only 84% - below typical levels for a company of this quality. The Strategic Documents analysis notes that the Board did not announce a defined multi-year buyback authorization at Investor Day, limiting the credibility of the capital return "pivot." The culture analysis identifies the dynastic structure as limiting independent challenge at the top. None of this is disqualifying, but it represents a governance discount that a fully independent structure would not carry.

Tensions between the analyses: The Investment Checklist gives 10 of 12 criteria a YES verdict and calls the valuation "attractive." The Red Flags analysis paints a materially more cautious picture, identifying recurring restructuring charges dressed as one-offs, buried impairments, and the

SERVE-HF historical precedent where a ResMed therapy actually increased mortality. The Earnings Call Deep Dive sits between these poles - confirming management's operational credibility on margins and guidance delivery while flagging the asymmetric engagement pattern (fluent on GLP-1, opaque on RCS, brief on Philips).

What the primary sources revealed that web research alone would have missed: The earnings transcripts exposed the progressive specificity of the GLP-1 counter-narrative and the conspicuous vagueness on RCS - patterns invisible in summary coverage. The Strategic Documents analysis revealed that the "500 million lives by 2030" goal implies 70M net patient adds per year against a current run rate of 15-20M - a gap never explained in any document. The Investor Day transcript showed management framing AI as a competitive moat before they had a commercial product to show for it. These are the kinds of insights that only come from reading the actual words.

Overall quality assessment: ResMed is a genuinely high-quality business - capital-light, high-margin, strong FCF generation, dominant market position, multi-decade runway. At ~22x trailing earnings and ~18x FCF, the valuation is reasonable for this quality. The key risk is not that the business deteriorates, but that the growth rate normalises from 10-12% to 6-8% as the Philips windfall fades and GLP-1 impact becomes clearer. At that growth rate, the current price is fair rather than cheap.

Next steps for deeper research:

- Read the Q3 FY26 earnings call (expected April 2026) for RCS trajectory and Philips re-entry commentary
 - Request and review the FY25 and FY24 proxy statements for detailed compensation metrics and goodwill impairment test assumptions
 - Model the Philips re-entry scenario: what does 5-8 points of device share loss look like on ResMed's P&L?
 - Track GLP-1 prescription volumes vs CPAP initiation rates quarterly to test management's "complementary" thesis
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Business History

ResMed's origin is less a conventional founding story than a technology transfer with a personal mission at its center. In 1987, Baxter Center for Medical Research Pty Limited acquired rights to CPAP technology that had emerged from the University of Sydney, where Professor Colin Sullivan first demonstrated continuous positive airway pressure as a non-invasive treatment for obstructive sleep apnea in 1980. Baxter began selling CPAP devices in Australia from 1988 but found no strategic fit within its larger portfolio. Dr. Peter Farrell, an engineer and entrepreneur who had been working at Baxter, recognized the clinical and commercial potential that Baxter was undervaluing. In 1989, he organized ResMed Holdings Pty Limited as an Australian entity specifically to acquire from Baxter both the CPAP technology rights and the existing device business - a management buyout alongside eight colleagues that became, in Mick Farrell's telling, the company's founding moment. As Mick described at Investor Day 2024, the origin was personal: his father walked into Sullivan's lab and encountered "patient one," Eddie Merck, who said the crude, noisy device had "saved my job, saved my marriage and saved my life." That encounter embedded a patient-centric ethos that has persisted across every subsequent strategic pivot.

The first decade was commercially modest and organizationally focused. ResMed Inc., a Delaware corporation, was incorporated in March 1994 as the ultimate holding company for the operating subsidiaries - a structural move that positioned the company for US capital markets. In June 1995, it completed an IPO on the NASDAQ National Market. During this period, as Mick Farrell acknowledged bluntly at the 2024 Investor Day, ResMed was primarily known as "the world's best CPAP mask company." Its bubble mask and subsequent range of patient interfaces defined its commercial identity, while rival Respirationics - the Pittsburgh-based company Mick referenced - dominated the flow generator market. The original business model was straightforward: sell hardware devices and masks through durable medical equipment distributors to patients diagnosed through sleep clinics. Revenue was largely one-directional: device sold, transaction complete.

The first meaningful strategic inflection came around 2005, driven by industrial design rather than technology. ResMed made a deliberate decision to compete on lifestyle aesthetics rather than clinical specifications alone, launching the S8 and then S9 series of flow generators. The framing was explicit: these were not medical equipment in the traditional sense but objects designed to coexist with a Pottery Barn bedroom set. This repositioning - making CPAP feel less clinical and more like a consumer product - expanded the patient base by reducing the stigma and discomfort that had suppressed adherence. By 2009, ResMed had become a genuine competitor in the flow generator segment it had previously ceded, and its market share in devices began closing the gap with Respirationics. In September 1999, prior to this device push, the company had transferred its principal listing to the New York Stock Exchange under the ticker RMD, followed in November 1999 by a secondary listing of Chess Depositary Instruments on the Australian Securities Exchange, maintaining its ties to its origins while accessing deeper US institutional capital.

The second, more consequential inflection arrived in 2014, and it amounted to a philosophical bet about what ResMed actually was. The company made a decision to embed cellular connectivity into 100% of its devices going forward, launching the AirSense 10 platform as the first cloud-connected CPAP at scale. CFO Brett Sandercock, as Mick recounted, raised the legitimate concern that cellular modems cost real money and had a calculable WACC drag: how would the return materialise? The thesis was what management later described as a "reverse Amazon play" - Amazon used cheap hardware (the Kindle) to unlock a high-value software and content ecosystem; ResMed would use its premium hardware base to unlock a high-value digital health platform. The company pre-mapped seven value-extraction pathways from connectivity and ultimately found more than it had anticipated. AirView, the cloud platform for clinicians to remotely monitor patient therapy compliance, became a genuinely sticky enterprise software product. The myAir patient app created a direct-to-consumer engagement layer that had not existed in the category. By building what became the world's largest dataset of sleep and breathing health - currently over 22 billion nights of de-identified breathing data - ResMed changed the structural economics of the category. Remote monitoring enabled DME providers to manage far more patients per clinical FTE, creating a measurable ROI that entrenched AirView as standard of care. CPAP adherence rates in digitally engaged patients exceeded 87%, versus historical industry averages well below 60%.

The software strategy then accelerated into adjacent territory through acquisition. In April 2016, ResMed acquired Brightree LLC for approximately \$800 million, acquiring a SaaS platform serving

home medical equipment and home infusion providers with billing, documentation, and business management tools. The logic was that ResMed's clinical devices and Brightree's operational software served overlapping customer sets - DME providers - and that owning both strengthened ResMed's position in the care coordination chain. This marked the official emergence of a second segment: what the company would come to call Residential Care Software. Two further acquisitions in 2018 deepened this segment: HEALTHCAREfirst in July 2018 (home health and hospice software) and MatrixCare in November 2018 for approximately \$750 million, adding skilled nursing facility, senior living, and life plan community platforms across six verticals. By the time these transactions closed, ResMed was generating meaningful SaaS revenue from providers who in some cases were not even ResMed device customers - a structural diversification away from the hardware cycle that had defined the original business.

The transition from Peter Farrell to Mick Farrell as CEO in 2013 - Peter remaining as Executive Chairman, then non-executive Chair from that year - had aligned with this strategic acceleration. The father had built the hardware franchise and the clinical credibility; the son, a PhD-trained engineer who had worked at McKinsey and at Intel before joining ResMed in 2000, brought a software-and-platform instinct to the leadership role. The dynastic element is unusual for a company of this scale but has proved operationally coherent: continuity of mission without stagnation in strategy.

The Philips Respironics recall, announced in June 2021 over foam degradation concerns in its CPAP and BiPAP devices, produced an extraordinary and unplanned structural advantage. Philips had been the category's co-leader, sharing the market roughly equally with ResMed. The recall removed Philips from new shipments to the US market for an extended period, during which ResMed absorbed a surge in demand - and, critically, converted millions of patients and providers to AirView and the connected ResMed ecosystem. By the time Philips began re-entering markets, Mick Farrell estimated at the Morgan Stanley Healthcare Conference in 2024 that Philips had been reduced to a third or fourth position in global sleep apnea and that its own five-year projections suggested a ceiling of roughly 20% market share on return - a fraction of its pre-recall position.

The most recent structural move extended the software footprint into Europe. In June 2022, ResMed announced the acquisition of MEDIFOX DAN, Germany's leading out-of-hospital software provider for home health and nursing home operators, at a price of approximately \$1 billion. At the M&A announcement call, Mick Farrell was explicit about the strategic rationale: MEDIFOX DAN was "the MatrixCare of Germany," built on recurring SaaS revenue at a 14% CAGR with over 8,000 customers, 300,000 caregivers on the platform, and immediate non-GAAP EPS accretion. The acquisition marked ResMed's first pure-play SaaS business outside the United States, and Mick framed it as validating the "RES" in ResMed as standing for residential - out-of-hospital care as the organizing mission of the enterprise, not merely sleep devices. The most recent acquisition, VirtuOx in May 2025 for \$140 million, extended the integration further upstream into diagnostics: an independent diagnostic testing facility offering at-home sleep, respiratory, and cardiac testing that now sits inside the care pathway that feeds patients toward CPAP initiation.

The cultural throughline connecting 1989 to 2025 is an orientation toward out-of-hospital care as both the right clinical setting and the most defensible commercial position. Every major strategic decision - connectivity, Brightree, MatrixCare, MEDIFOX DAN, VirtuOx - has reinforced that single vector. The company Peter Farrell founded to sell masks around the world has become a company that manages

the entire clinical and operational infrastructure of out-of-hospital chronic disease care, with devices as the patient-facing endpoint and software as the margin-expanding, sticky layer underneath.

Management & Incentives

Michael "Mick" Farrell has been CEO since March 2013 and Chairman since January 2023, combining both roles. His twelve-plus years at the helm represent one of the longer tenures among large-cap medtech CEOs, and his background is unusually deep within the company: he joined ResMed in the late 1990s, ran the Americas business from 2011-2013, and held senior roles in marketing and business development before that. His academic credentials (BS Engineering from University of New South Wales, MS Chemical Engineering and MBA from MIT Sloan) align with a technically grounded, operationally minded leader. He is also Chair of AdvaMed's board as of January 2026, which raises modest time-allocation questions but signals meaningful industry influence.

Brett Sandercock has served as CFO since January 2006 - making him a rare two-decade incumbent in the role. He came up through ResMed's finance function (group accountant from 1998, VP Treasury and Finance from 2004) after earlier stints at Saint-Gobain and PricewaterhouseCoopers. His deep institutional knowledge of the company's financial architecture is a genuine asset; it also creates key-person concentration at the CFO level. On earnings calls, the Farrell/Sandercock pairing displays a practiced dynamic: Farrell leads on strategy and narrative, Sandercock handles the numbers crisply and without inflation. In Q&A on the Q2 FY26 call, both handled analyst pushback on SG&A growth and VirtuOx contribution clearly and specifically, without deflection.

The Founder Dimension. Peter Farrell - Mick's father - founded ResMed in 1989 and served as Chairman until January 2023, when he transitioned to Chairman Emeritus while remaining on the board as a non-independent director. The dynastic character of this governance is the most notable structural feature of ResMed's management profile. Father built the company; son runs it. Both sit on the board. This is not inherently disqualifying - Mick has proven his operational credentials over more than a decade - but it creates layers of principal-agent ambiguity that a fully independent board would not permit. The board appears to have deliberately managed this by installing Ron Taylor (founder and CEO of Pyxis Corporation, a 20-year director) as Lead Independent Director, a role that becomes critical precisely because the Chairman/CEO combined role is held by a family insider.

Insider Ownership and Skin in the Game. Total insider ownership sits at approximately 0.5-0.56% of shares outstanding, worth roughly \$170-\$210 million at recent prices. Mick Farrell personally holds approximately 468,000 shares as of March 2026, worth around \$103 million at current prices - a meaningful stake for a CEO but modest relative to the company's \$32 billion market cap (roughly 0.3%). Farrell's entire trading activity over the past five years consists of 45 sells and zero open-market buys, all conducted under 10b5-1 plans. This pattern is common and legally clean, but investors should note that no CEO-level buying has occurred on dips. The absence of insider buying during periods of price weakness - most notably the 2023 GLP-1 selloff when RMD fell roughly 40% - is a mild negative signal on conviction.

Compensation Structure. Mick Farrell's total compensation was approximately \$14.7 million in FY2025, up roughly 4% from \$14.1 million in FY2024. The proxy states that 91% of his direct

compensation is "at risk" in the form of incentive compensation, with the remaining 9% representing base salary (approximately \$1.1 million). Other named executive officers average 81% at-risk compensation. The pay structure is heavily weighted toward long-term equity - consistent with medtech peer practice. The STI plan uses operational metrics (the proxy references non-GAAP operating profit and revenue growth as the primary drivers), and both the STI and LTI can pay out at zero in a down scenario. A formal Compensation Recovery (clawback) policy, adopted in October 2023, allows recovery of incentive compensation following a financial restatement even absent misconduct. The CEO-to-median-employee pay ratio of approximately 187:1 is within normal range for a global medtech company. The FY25 say-on-pay advisory vote passed with approximately 84% support - not overwhelmingly high.

Capital Allocation Track Record. Over the three years through FY25, ResMed deployed capital roughly as follows: 27% to R&D, 33% to M&A, 31% to dividends, and 9% to buybacks. On the positive side: the Brightree acquisition (2016, ~\$800M) integrated cleanly and has become a strategic moat in the US HME software market. The dividend has increased consistently; the FY26 annualised rate of \$2.40/share represents a 13% increase year-on-year. The \$600M+ buyback commitment for FY26 is credible given \$1.4 billion of cash on the balance sheet and FCF generation north of \$1.6 billion in FY25. The MEDIFOX DAN acquisition (~\$1 billion) is the more complicated case. The deal entered at roughly 27x LTM EBITDA. Post-acquisition, the broader RCS business has encountered persistent headwinds in the senior living and long-term care verticals, with management deferring recovery to FY27.

Board Composition. The board currently has eleven members, nine of whom are independent. Notable independent directors include Ron Taylor (Lead Independent Director, 20-year tenure), Christopher DeOrefice (Audit Committee Chair, CFO of Ulta Beauty, joined August 2024), Nicole Mowad-Nassar (SVP AbbVie, joined August 2025), and Desney Tan (former Microsoft Research Managing Director - technology and AI expertise). The board refresh trajectory is positive: two new independent directors in the past 18 months.

Does Management Do What It Says? The track record is notably clean. When Farrell flagged on the Q1 FY26 call that he expected EMEA/APAC masks to reaccelerate to high-single-digit growth in Q2, the team delivered 8% constant-currency growth - exactly as guided. The supply chain gross margin improvement program has delivered consistently ahead of investor expectations. The one area where delivery has lagged stated ambition is the RCS software segment. However, Farrell has been consistent in acknowledging the miss and resetting expectations rather than papering over it.

Competitive Landscape

The obstructive sleep apnea (OSA) device market is a concentrated duopoly that has, for the past several years, functioned as a near-monopoly. ResMed and Philips Respironics historically commanded roughly 80%+ of global sleep apnea device revenue together. Since June 2021, Philips has been absent from the market following a massive voluntary recall of its PAP devices due to polyurethane foam degradation that posed inhalation and potential carcinogenic risks to users. A consent decree finalised by a federal court in April 2024 restricts Philips from resuming CPAP and

BiPAP sales in the United States until it can demonstrate sustained manufacturing quality and sufficient inventory to meet domestic demand. As of early 2026, Philips has not re-entered the US device market, though it continues to sell masks in some geographies and has been slowly re-entering select international markets.

The result is a market in which ResMed holds an estimated 55-60% global share of sleep apnea devices, with Fisher & Paykel Healthcare (New Zealand) occupying a distant third position, primarily in masks and humidification accessories. The overall sleep apnea devices market is valued at approximately \$6.9 billion in 2025 and is projected to reach \$11.6 billion by 2032, implying a ~7.6% CAGR driven almost entirely by diagnosis rate expansion, not share-shifting among incumbents. Inspire Medical Systems (INSP) represents a structurally distinct competitor. Its hypoglossal nerve stimulator is a fully implantable surgical device for CPAP-intolerant patients with moderate-to-severe OSA. Inspire holds over 86% of the nascent implantable neurostimulation segment. Critically, Inspire targets only the CPAP-intolerant subset - CPAP failures eventually become Inspire candidates, not vice versa.

Competitive Advantages and Their Durability. ResMed's moat is multi-layered and reinforcing, built on intangible assets, switching costs, and a data network effect that is now approaching the scale needed to become self-compounding.

Intangible assets and clinical evidence: ResMed has over three decades of regulatory approvals, clinical trial data, and physician relationships. Its AirSense device family is the reference standard in most sleep lab and home care protocols. In December 2025, ResMed received FDA clearance for Smart Comfort, the first AI-enabled CPAP device to recommend individualised pressure settings - extending its product differentiation lead at precisely the moment Philips is attempting a re-entry.

Digital platform and data flywheel: The AirView cloud platform manages more than 32 million patients, supported by over 26 million connected devices and 22 billion nights of respiratory data. This dataset is growing faster than any plausible competitor can replicate. The myAir patient engagement app, combined with the "Dawn" AI concierge, increases therapy adherence and creates a bidirectional data loop. At the Investor Day 2024, management explicitly described this as "shifting the basis of competition yet again with a tangible AI and data-driven competitive advantage." The platform also integrates upstream (Brightree, MatrixCare, Somnoware for provider and home health workflows) and downstream (resupply automation), creating meaningful provider and HME switching costs.

Manufacturing and scale: ResMed benefits from learning curve advantages in miniaturised electronics, CPAP flow generators, and mask manufacturing. CPAP devices are excluded from CMS competitive bidding, which protects margins from race-to-the-bottom pricing dynamics. Medical device tariff exemptions provide additional cost stability.

These advantages are durable but not impervious. The primary erosion risk is Philips' eventual return to the US market. Philips retains manufacturing capabilities, a recognised brand among prescribers, and institutional relationships with large IDNs and DME distributors. Management commentary across multiple earnings calls suggests Philips has been price-aggressive in European and Australian markets where it has already re-entered, though ResMed has maintained and in some geographies grown share despite that pressure.

GLP-1 Drugs: Threat Reassessment. The FDA's December 2024 approval of tirzepatide (Zepbound) for OSA treatment in obese adults was widely cited as a potential disruption to CPAP demand.

However, the net demand picture is nuanced. ResMed's own data shows GLP-1 users are 10-11%

more likely to initiate CPAP and show higher long-term adherence - likely because engaging the healthcare system for obesity treatment creates a pathway to OSA diagnosis that would otherwise be missed. GLP-1 adherence rates for weight loss are poor - most patients discontinue within three to four months - and even patients who remain on GLP-1 drugs for over a year under controlled conditions typically transition from severe OSA to moderate OSA, not to full remission. With 1.4 billion estimated OSA sufferers globally and penetration rates of 15-20% in the US and under 5% in APAC, the diagnosis gap is so vast that GLP-1-driven demand attrition at the margin is swamped by secular expansion in newly diagnosed patients.

Barriers to Entry. FDA Class II 510(k) clearance is required in the US. Developing the cloud telemetry infrastructure, compliance reporting tools, and payer integration required to compete for institutional DME provider contracts takes years and hundreds of millions in development spend. Chinese manufacturers (BMC, DeVilbiss) have entered the low-cost segment and are gaining ground in APAC but lack the clinical credibility and data infrastructure for the US and Western Europe.

Position in the Value Chain. ResMed has deliberately expanded its position across the entire OSA value chain: from population health screening (NightOwl, Somnoware) to device therapy and masks, to patient engagement and adherence management (AirView, myAir, Dawn AI), to provider workflow and billing software (Brightree, MatrixCare), to mask resupply within its own ecosystem. This vertical integration is unusual for a medical device company and creates a structural advantage: ResMed earns revenue at each stage of the patient journey and creates data feedback loops across all of them.

Financial Characteristics

ResMed's financial profile is distinctive within medical technology: a high-gross-margin consumables-driven business with near-zero capital intensity, exceptional cash conversion, and a balance sheet that has swung to net cash - all while growing revenue at a double-digit pace.

Revenue Model and Mix. ResMed generates revenue through three streams. Devices (flow generators, CPAP and bilevel machines) contributed approximately 51-52% of FY25's \$5.146B in revenue and are the highest unit-value item per sale, though they carry lower gross margins than consumables. Masks and accessories - the recurring consumable layer - accounted for roughly 35-37% of revenue. This is structurally the most valuable segment: masks require replacement every three to six months, create a predictable annuity stream, and carry margins materially above corporate average. Residential Care Software (principally Brightree and MatrixCare) contributed approximately 12%, a stable, high-margin SaaS-like layer. Geographically, the US, Canada, and Latin America generate approximately 60% of revenue, with Europe, Asia, and the rest of the world accounting for the remaining 40%.

Margins: Levels and Trajectory. Gross margin has been expanding meaningfully. FY24 gross margin was 56.7%; FY25 reached 59.4%, a 270-basis-point improvement. The most recent quarters show continued momentum: Q4 FY25 at 60.8%, Q1 FY26 at 61.5%, and Q2 FY26 at 61.8%. Management's FY26 guidance calls for 62-63% gross margin. Drivers include mix shift toward higher-margin masks and software, manufacturing efficiencies from scale, and the structural benefit of

Philips' sustained absence from the market.

Operating margin expanded from 28.2% in FY24 to 32.7% in FY25 - a 450-basis-point improvement. FY26 guidance implies SG&A at 19-20% of revenue and R&D at 6-7%, suggesting operating margins sustaining above 30%. For context, the broad medtech sector averages EBITDA margins in the 20-22% range; ResMed's FY25 operating margin at 32.7% places it in the top tier globally. Net margin reached 27.2% in FY25.

Returns on Capital. ROIC improved sharply. Using operating equity plus long-term debt less cash as the capital base: FY24 ROIC was approximately 19.2% (\$1,021M net income on \$5,323M invested capital); FY25 ROIC improved to approximately 25.9% (\$1,401M on \$5,417M). On a tangible capital basis - stripping out goodwill (\$3.047B) and intangibles (\$465M) - ROIC exceeds 60%, reflecting the reality that the underlying business requires very little physical capital to operate. ROE moved from 21.0% in FY24 to 23.5% in FY25.

Cash Conversion. In FY25, CFO was \$1.752B against net income of \$1.401B, a cash conversion ratio of 125%. FCF (after \$90M capex) was \$1.662B, a FCF/NI ratio of approximately 119%. LTM operating cash flow as of Q2 FY26 reached \$1.91B. This superior conversion reflects non-cash charges (D&A on acquired intangibles) that inflate GAAP COGS but generate real cash, and the recurring consumables model producing stable, forecastable collections.

Balance Sheet Strength. Net debt moved from approximately \$459M at end-FY24 to net cash of \$551M at end-FY25 (\$1.209B cash against \$658M LT debt). Total equity grew from \$4.864B to \$5.968B. Goodwill (\$3.047B) and intangibles (\$465M) represent \$3.5B, or roughly 43% of total assets - largely the legacy of software acquisitions - and are a risk factor to monitor.

Capital Intensity. Capex was \$99M in FY24 and \$90M in FY25, representing just 1.7% of revenue. The majority is likely maintenance and tooling rather than growth capex. This compares favourably even to software businesses.

Working Capital Dynamics. DSO was approximately 66 days in FY25, consistent with FY24 (65 days). Days inventory on hand was elevated at roughly 162 days in FY25, up from 148 days in FY24. The inventory build partly reflects strategic safety stock decisions. This is the primary working capital tension in the model.

Cyclicality. ResMed's business is structurally defensive. Sleep apnea is a chronic condition requiring continuous therapy; once patients are initiated on CPAP, mask replenishment is largely non-discretionary and is often reimbursed. Revenue showed no cyclical sensitivity during COVID; demand actually accelerated.

Red Flags

Accounting Quality: Non-GAAP Theatre and the Restructuring Habit

The most persistent accounting concern at ResMed is the company's reliance on non-GAAP reporting. Management routinely strips out amortisation of acquired intangibles, restructuring charges, acquisition costs and field safety notification expenses when presenting "adjusted" results. The problem is that these are not genuinely one-off items - they are recurring features of the business model.

Restructuring charges have appeared in nearly every fiscal year for a decade. In FY24 alone, the company recorded \$64 million in restructuring-related charges, comprising \$28.6 million of employee severance and \$33.2 million of intangible asset impairments tied to wind-downs of business activities. That was followed by \$16 million in Q1 FY26 and \$21.7 million in Q2 FY26. When a company strips out costs year after year and calls them non-recurring, the investor is effectively being asked to value a company on earnings that the business cannot actually sustain. Goodwill is now \$3.047 billion on \$8.174 billion of total assets - 37% of the balance sheet. ResMed has never taken a meaningful goodwill impairment charge, but the Residential Care Software segment is already exhibiting stress. Management in Q2 FY26 acknowledged "mid-single-digit growth" for the remainder of FY26. Given the company paid ~\$1.0 billion for MEDIFOX DAN at a significant premium to book, any sustained underperformance creates latent impairment risk. The precedent from FY24 is instructive: \$33.2 million of intangible impairments were buried inside a restructuring charge rather than disclosed as a standalone impairment.

Governance: The Farrell Dynasty Problem

ResMed presents a textbook founder-family entrenchment dynamic. Peter Farrell founded the company, served as CEO and Chairman for decades, and his son Mick Farrell now holds both the CEO and Board Chairman roles simultaneously. The combined Chair/CEO structure is a governance red flag under virtually any framework - it removes a critical check on management, concentrates agenda-setting power, and creates an obvious conflict in evaluating CEO performance. The more acute governance risk sits in capital allocation. Under Mick Farrell's tenure, ResMed has acquired VirtuOx, Somnoware, EctoSense's Nidal product and has announced plans for continued M&A. Each deal is small, but the pattern is empire-building through acquisition. Investors should ask whether any of these software acquisitions has ever generated a documented, post-acquisition return on invested capital that exceeded the cost of capital.

Regulatory and Legal Exposure

The DoJ/OIG kickback settlement (\$37.5 million payment and Corporate Integrity Agreement) deserves more investor attention. The government alleged that ResMed provided free goods and services to DME suppliers, sleep labs and other healthcare providers as inducements to increase referrals. The company also executed a Class I FDA recall in 2023-2024 covering more than 20 million masks fitted with magnets due to potential interference with cardiac implants. At least six injuries requiring hospitalisation were reported.

The SERVE-HF Warning

The 2015 SERVE-HF trial showed that Adaptive Servo-Ventilation therapy actually increased cardiovascular mortality by 2.5 percentage points per year in heart failure patients with low ejection fraction. The study was halted early on safety grounds. The bear case today rhymes with SERVE-HF: CPAP's clinical evidence for hard cardiovascular outcomes is weaker than the company's marketing implies. If a well-powered clinical trial were to show no significant effect on cardiovascular mortality, the reimbursement environment could shift materially.

The GLP-1 Risk: Dismissed Too Quickly

The GLP-1 data cited by management is observational and self-selected: patients motivated enough to seek both obesity treatment and sleep apnea diagnosis are a healthier, more engaged cohort than the average OSA patient pool. The real long-term question is whether a sustained, population-wide reduction in obesity prevalence will compress the incident case flow into the CPAP funnel five to ten years from now.

Questions an Investor Must Ask Management

1. What is the carrying value and WACC assumption used in the annual goodwill impairment test for the Residential Care Software reporting unit?
2. Is the "mid-single-digit growth" guidance for RCS a function of deliberate portfolio pruning, or genuine volume deceleration from competitive pressure?
3. Under the Corporate Integrity Agreement, what specific sales practices were prohibited, and how does management ensure the commercial incentive structure does not recreate the behaviour that led to the settlement?
4. What is management's honest estimate of the incremental revenue captured as a result of the Philips recall?
5. In the FY24 restructuring that included \$33.2 million of intangible impairments, which specific assets were written down, and why was this disclosed inside a restructuring line item?
6. For MEDIFOX DAN, what is the post-acquisition revenue CAGR relative to the original acquisition underwriting model?

The bear case in a sentence: Philips re-enters the market and recaptures 5-8 points of device share; GLP-1s begin compressing incident OSA diagnosis rates within 5 years; the software segment requires a goodwill write-down; and the combined Chair/CEO structure ensures no independent check on management's response to all three simultaneously.

Investment Checklist

Stock price: ~\$221 | Market cap: ~\$32.6B | Trailing P/E: ~22x | Forward P/E: ~19x | EV/EBITDA: ~15.8x | EV/FCF: ~17.8x

- 1. Understandable Business - YES.** ResMed makes the devices, masks, and software that treat sleep-disordered breathing. A patient is diagnosed, prescribed a CPAP device, wears a mask every night, and resupplies consumables on a recurring basis. The cloud platform AirView sits on top, generating adherence data. The business model is device placement plus durable, recurring consumables - conceptually identical to the razor/razor-blade model.
- 2. Durable Competitive Advantages (Widening, Not Narrowing) - YES.** ResMed holds approximately 62% global market share in CPAP devices. AirView (32M+ monitored patients, 22B+ nights of data) creates a proprietary data asset. FDA regulatory approval, long device qualification cycles, and physician familiarity all raise switching costs. Gross margins expanding from 56.7% in FY24 to 61.8% in Q2 FY26 are the clearest evidence that the moat is widening.
- 3. Long Reinvestment Runway at High Returns - YES.** OSA affects 1.4 billion people globally; US penetration is only 15-20%. The undiagnosed population represents a multi-decade growth runway. Adjacent expansion into out-of-hospital software opens an additional market.

4. High Returns on Incremental Capital - YES. FY25 ROE of ~23.5%. Capex of only \$90M against FCF of \$1.66B represents a ~5% capex-to-FCF ratio. Operating margins expanded nearly five percentage points year-over-year.

5. Strong Free Cash Flow Generation - YES. FCF of \$1.662B in FY25, a 28% increase. FCF margin on revenue approximately 28%. The FCF yield on current market cap is roughly 5%.

6. Conservative Balance Sheet - YES. Net cash position of \$541M. The balance sheet risk worth monitoring is goodwill of \$3.047B (37% of total assets). However, with \$1.66B in annual FCF, even a full goodwill impairment would not threaten business continuity.

7. Aligned Management with Skin in the Game - PARTIALLY. Mick Farrell owns approximately 468,000 shares worth ~\$103M. However, 45 sells and zero purchases over five years, including six sales in the past six months. No open-market buying undermines the "skin in the game" narrative. The dynastic element provides strategic continuity as a partial offset.

8. Rational Capital Allocation History - YES. 27% R&D, 33% M&A, 31% dividends, 9% buybacks. Brightree and MatrixCare extended the addressable market. FY26 buybacks tracking at \$600M+. No history of dilutive equity issuances or vanity deals.

9. Favourable Industry Structure - YES. Effectively a duopoly with Philips impaired. CPAP excluded from CMS competitive bidding. Consumables generate reorder economics with high retention. Barriers to entry are high.

10. Reasonable Valuation Relative to Quality and Growth - YES. At ~22x trailing earnings, 19x forward, 15.8x EV/EBITDA - below its five-year average EV/EBITDA of ~26.5x. For a business growing at 9-11% organically with expanding margins and \$1.66B+ in FCF, this is materially below-average. Analyst consensus target averages ~\$290-297.

11. Resilience to Recession or Disruption - PARTIALLY. Sleep apnea is chronic and demand is inelastic. The primary disruption risk is GLP-1 medications. The FDA approved tirzepatide for moderate-to-severe OSA in late 2024. Near-term data is constructive, but the long-term disruption tail requires monitoring.

12. Transparent Management Communication - YES. Management has been proactively forthcoming on GLP-1 with real-world data. Gross margin guidance has been specific and credible. No history of guidance sandbagging or restatements.

Overall Summary: ResMed passes eleven of twelve criteria outright and earns a partial on two (management skin-in-the-game and disruption resilience). At ~22x earnings and ~18x FCF for a business of this quality, the entry point is attractive. The primary risks to monitor are GLP-1 erosion, goodwill impairment from software acquisitions, and the pace of Philips' re-entry. For a long-term investor, ResMed represents the rare combination of durable competitive advantage, visible growth runway, strong capital returns, and a reasonable price.

Earnings Call Deep Dive

Based on systematic analysis of the Q3 FY25, Q4 FY25, Q1 FY26, and Q2 FY26 earnings call transcripts.

Narrative Evolution. Across the four quarters, management's core storyline has remained consistent - a long-duration structural growth thesis around under-diagnosed OSA - but the way they've packaged and defended it has shifted meaningfully.

In Q3 FY25, Mick Farrell was in an almost euphoric posture. The Lancet meta-analysis showing CPAP reduces all-cause mortality by 37% gave him a clinical anchor to double down on. The tone was expansive, almost promotional.

By Q4 FY25, the narrative had rotated toward capital discipline and durability. Farrell introduced the "500 million lives by 2030" goal more formally, and Brett Sandercock anchored the call in margin expansion. Leadership also spent more time on GLP-1 in this call than any prior quarter, preemptively framing it as a net positive before analysts had fully pivoted from fear to curiosity on the topic. Q1 FY26 was the weakest quarter in the sequence, and the tone shifted noticeably. Revenue came in at \$1.34B (+9%), NI declined sequentially, and the \$16M restructuring charge required explaining. Farrell's language grew more carefully hedged - the OSA prevalence "projected to rise 35% by 2050" was a new long-range stat deployed as a confidence anchor when near-term numbers were softer. Notably, management introduced "FY27" as the target for RCS to return to high-single-digit growth - a meaningful deferral.

Q2 FY26 was the strongest quarter (revenue \$1.42B, +11%; GM 61.8%), and the tone swung back to offense. Farrell led with FDA clearance for the AI comfort recommender and 60,000 CME completions. The restructuring charge grew to \$21.7M, and management was tighter-lipped about what specifically was being restructured.

Guidance and Delivery. Management's guidance has been precise and they've delivered consistently above the midpoint on margins. Q3 FY25 guided for Q4 gross margin "broadly consistent" with Q3's 59.3-59.9%. Q4 came in at 60.8% - well above "broadly consistent." For FY26, gross margin of 61-63% is tracking toward the upper half after just two quarters. Sandercock has not tightened the range despite tracking above midpoint - either discipline or deliberate under-promise. The one guidance item that has slipped is RCS.

Analyst Concerns. Three topics dominate Q&A:

GLP-1 and OSA prevalence - the most-asked topic. Management's response has evolved from defensive to offensive ("GLP-1 users are 10-11% more likely to start CPAP"). No analyst has accepted the narrative as closed.

Residential Care Software deceleration - RCS grew 10% in Q3 FY25, then dropped to 5-6% in Q1-Q2 FY26. Management's answers focus on "leadership transition" and "strategic realignment" - notably vague for a 12% of revenue segment.

Philips re-entry - analysts asked about competitor re-entry in international markets. Farrell's response was confident but short on operational detail.

Key Reveals:

1. *"Real-world data shows patients on GLP-1s are 10-11% more likely to start CPAP therapy and have higher long-term adherence."* (Q2 FY26) - The most significant data point on GLP-1 across all four calls, flipping the narrative from threat to tailwind.

2. *"Residential Care Software expected to return to high single-digit growth and double-digit operating profit growth in FY27."* (Q2 FY26) - The explicit deferral of RCS recovery implies either headcount rationalisation or a confidence level management hasn't publicly grounded in specifics.

3. *"We published a landmark meta-analysis in The Lancet showing CPAP therapy reduces all-cause mortality by 37%."* (Q3 FY25) - A significant positioning shift with regulatory and reimbursement implications.

4. *"CPAP, APAP, and bilevel products are excluded from CMS competitive bidding."* (Q2 FY26) - Stated as fact rather than policy position, with no caveats about review cycles.

5. *Indianapolis distribution center and Calabasas manufacturing opening* - Management is systematically reducing geographic concentration risk in US operations, a strategic shift that has

received almost no analyst attention.

Q&A Dynamics. The most telling pattern: management engages fluently on GLP-1, gives boilerplate on RCS, and is brief on Philips. The asymmetric engagement - substantive on strengths, opaque on weaknesses - is the clearest signal from the transcript record.

Strategic Documents Analysis

*Based on analysis of the Investor Day 2024, Corporate Presentation Sep 2025, JP Morgan

Healthcare Conference 2026, and 10-K FY25.*

Strategic Priorities. ResMed's 2030 strategy rests on three explicit pillars. First, defend and expand the sleep apnea core through demand generation, AI-enabled product innovation, and superior adherence data. Second, capitalise on adjacencies - scaling into broader sleep health and breathing health categories, including COPD and out-of-hospital care software. Third, build a fully integrated digital ecosystem - investing 7% of revenue in R&D to deepen the AirView/myAir data moat and embed AI/ML across therapy management.

The evolution from prior messaging is meaningful. In 2022-23, with Philips mired in recall chaos, ResMed's narrative was predominantly opportunistic - capture displaced patients. By late 2024, the rhetoric shifted toward penetration of the untreated pool rather than competitive share gains, reflecting a mature-share story with the Philips tailwind largely exhausted. At the January 2026 JP Morgan Healthcare Conference, Farrell added a new emphasis on AI-enabled therapy personalisation, specifically the "Smart Comfort" AI device launching in limited beta.

Capital Allocation Framework. Management's stated hierarchy is: organic reinvestment first, then disciplined M&A, then balanced capital return. The numbers mostly support the rhetoric. A significant shift is now appearing in capital return posture - after years of minimal buyback activity, ResMed announced plans to repurchase more than \$600 million in FY26, a near step-change. This pivot coincides with FCF running at \$1.66 billion and a balance sheet that requires no defensive hoarding. But it also reflects an absence of large-scale M&A targets, not purely conviction-driven capital return.

Long-Term Targets. At the 2024 Investor Day, ResMed issued a five-year financial outlook:

high-single-digit revenue CAGR (7-9%), earnings growth exceeding revenue growth, gross margin 61-63%, SG&A 19-20%, R&D ~7%. These targets are credible and the company is executing ahead of them. Revenue growth of 9-11% in FY26 sits at the top of the "high single digit" framing. The omission of an explicit operating margin target is a telling gap - management is deliberately leaving room to invest more aggressively if required.

Narrative vs Reality. Where the narrative aligns: the gross margin expansion story is real and ahead of schedule. The data moat framing (32M+ patients, 22B nights of data) is real infrastructure. The GLP-1 risk-reframing is substantiated by ResMed's own data.

Where the narrative diverges: the residential care software segment tells a more complex story. At roughly 12% of revenue and holding flat as a proportion over three fiscal years, it has not become the second growth engine that billion-dollar acquisitions promised. The JP Morgan 2026 presentation leaned heavily on AI and connected care without providing explicit revenue attribution to software platforms.

What is conspicuously absent: pricing power is never explicitly addressed. A company genuinely confident in pricing power would not lead with relief at regulatory forbearance. Management also never

explicitly quantifies how much of FY2022-FY2024 outperformance was Philips-driven versus structural penetration.

Addressable Market Framing. Management defines TAM as 1.4 billion people globally with OSA, against US penetration of only 15-20%. The core framing is credible in absolute scale but analytically vague. The 500-million-lives goal by 2030 from 154 million currently would require adding roughly 70 million patients per year - against a current base suggesting 15-20 million net adds annually. That gap is not explained in strategic documents, making the mission goal inspirational rather than operational.

Risk Acknowledgement. Strategic documents acknowledge GLP-1 risks more candidly than typical, but the framing is consistently asymmetric. GLP-1s are "complementary." Competitive bidding exclusion is a "great win." Philips' return is "game on." Reimbursement risk is the most underacknowledged threat in all documents reviewed.

Corporate Culture

Founder Imprint. Peter Farrell built ResMed from a single insight - that sleep apnea was chronically underdiagnosed and underserved. Trained as a chemical and biomedical engineer at Sydney University, MIT, and the University of Washington, Farrell was fundamentally a problem-solver who believed "without engineering, medicine is just not going to advance at the level that it should." His engineering rigour became the company's operating philosophy. The transition to Mick Farrell in 2013 was one of the more seamless founder-successions in medtech. The leadership team is strikingly long-tenured (CFO Brett Sandercock 25 years, Chief Revenue Officer Mike Fliss 23 years, CPO Vered Keisar 19 years), which is unusual in a \$32B medtech and more consistent with mission-driven retention.

Mick Farrell has added his own inflection, running the company with the philosophy "only the paranoid survive" and publicly stating that firing three senior executives for not living up to core values was among his hardest decisions - but a necessary one.

Decision-Making Style. A hybrid: deliberately centralised at the strategic level, with Mick Farrell driving a coherent single mission across all business units, but operationally decentralised across the acquired software portfolio. The Philips recall response showed strategic clarity and supply chain creativity, but also the limits of manufacturing agility - they acknowledged missing approximately \$100 million in potential profit due to supply constraints.

Talent and Retention. Glassdoor shows approximately 4.0/5.0 overall, with 76% willingness to recommend. Average global voluntary turnover for FY2025 was approximately 12%. Counter-evidence comes from review narratives: recurring criticisms mention middle management as "political," internal competition producing "half-baked solutions," and abrupt handling of the FY2024 restructuring when ResMed cut approximately 5% of its workforce.

Innovation Culture. ResMed runs a dual-track model: sustained internal R&D focused on devices and CPAP connectivity, alongside acquisition-driven software expansion. The company invested heavily in AI before the current boom, which is more credible than companies now claiming AI-first credentials. The MEDIFOX DAN integration represents the cultural stress point - integrating a 700-person German software company is inherently difficult.

Integrity and Ethics. The 2020 False Claims Act settlement (\$37.5 million) to resolve five whistleblower lawsuits alleging kickbacks to DME suppliers casts a shadow. The company entered a five-year Corporate Integrity Agreement. The absence of subsequent enforcement action is mildly reassuring.

Red Flags. The leadership team's exceptional stability carries a latent risk: insiders who have never experienced failure at ResMed may lack cognitive diversity to anticipate disruption. The dynastic governance structure limits independent challenge at the top. The Glassdoor signals on mid-level political behaviour and poor communication during restructuring are consistent with what happens when high-growth companies accumulate management layers.

Learning Resources

Podcasts & Audio

1. **"Making Money While the World Sleeps - ResMed" | Equity Mates Investing Podcast** - Australian investor-focused deep dive covering the razor-and-blade business model and the massive underdiagnosis opportunity. equitymates.com
2. **"ResMed CEO Mick Farrell Balances Career, Family, and Bringing Tech, Software and Data into Medtech" | Medtech Talk Podcast** - Mick Farrell traces his path and explains the pivot from hardware to digital health and SaaS. medtechmvp.com
3. **"The Evolution of ResMed with Mick Farrell" | The Post-Acute POV Podcast** - Covers ResMed's growth from pure sleep device maker into a global leader in out-of-hospital software. audible.com
4. **"Losing Sleep and Lives: Matthew Walker on the Global Sleep Epidemic" | RTW Investments Podcast** - UC Berkeley sleep scientist makes the case for the scale of the sleep health crisis. rtwfunds.com

YouTube & Video

1. **ResMed 2024 Investor Day Webcast** - The September 2024 NYSE event where management unveiled the 2030 Strategy. investor.resmed.com
2. **ResMed Q2 FY26 Earnings Webcast (Jan 29, 2026)** - Most recent earnings call with 11% revenue growth and strong margin expansion. investor.resmed.com
3. **ResMed CEO Mick Farrell One-on-One with Jim Cramer (April 2024)** - Concise coverage of the GLP-1 drug threat and digital health positioning. cnbc.com
4. **ResMed Events & Presentations Archive** - Full archive of past webcasts including JP Morgan Healthcare Conference presentations. investor.resmed.com

Books

1. *****Why We Sleep: Unlocking the Power of Sleep and Dreams*** - Matthew Walker (2017)** - The foundational text for understanding sleep deprivation as a public health crisis. Makes the demand-side case for why ~936M people globally have sleep apnea. Amazon
2. *****The Innovator's Prescription: A Disruptive Solution for Health Care*** - Clayton Christensen et al. (2009)** - The defining framework for how medical device companies move up the value chain into software, data, and services - precisely the transformation ResMed executed.

3. *****The Sleep Revolution*** - Arianna Huffington (2016)** - Broader cultural argument for sleep as a health imperative that has expanded ResMed's addressable market.

Long-Form Articles & Research

1. **"ResMed: Medical Devices For Sleep Apnea By Leading Company" | LongTermPick** - Comprehensive investment deep dive covering business model, competitive moat, and valuation. longtermpick.com
2. **"ResMed, Inc.: The Shift to Connected Medical Devices" | IMD Business School Case Study** - Premier academic case examining the transformation from product company to health informatics business. imd.org
3. **"Could GLP-1 Drugs Make ResMed's CPAP Machines Obsolete?" | The Motley Fool (Nov 2025)** - Thorough treatment of the key bear thesis and why ResMed's data suggests otherwise. fool.com
4. **"Shifting Tides in Sleep Apnea Market" | Jefferies** - From Philips re-entry to Ozempic, analysis of competitive dynamics. jefferies.com

Earnings Calls & Primary Sources

- **Quartr** - All transcripts, slides, and audio for ResMed events. quartr.com/companies/5574
- **ResMed Investor Relations** - Official hub for 10-K filings, press releases, and webcasts. investor.resmed.com
- **ResMed 2030 Strategy Press Release** - investor.resmed.com

Suggested Learning Path

Start with Matthew Walker's **Why We Sleep** to internalise the demand-side thesis. Then listen to the Equity Mates episode for a clean investor-friendly overview. Move into the LongTermPick deep dive to build your financial framework, and read the Motley Fool GLP-1 article to stress-test the bear case. Then go directly to primary sources: pull up the ResMed 2024 Investor Day on Quartr to hear management articulate the 2030 Strategy, then work through the Q2 FY26 earnings call transcript to track execution. The IMD case study is a useful companion for understanding the software pivot intellectually.

Analysis generated using Quartr primary-source data (transcripts, financial statements, strategic documents) combined with web research. All financial figures sourced from Quartr's structured data and ResMed SEC filings.